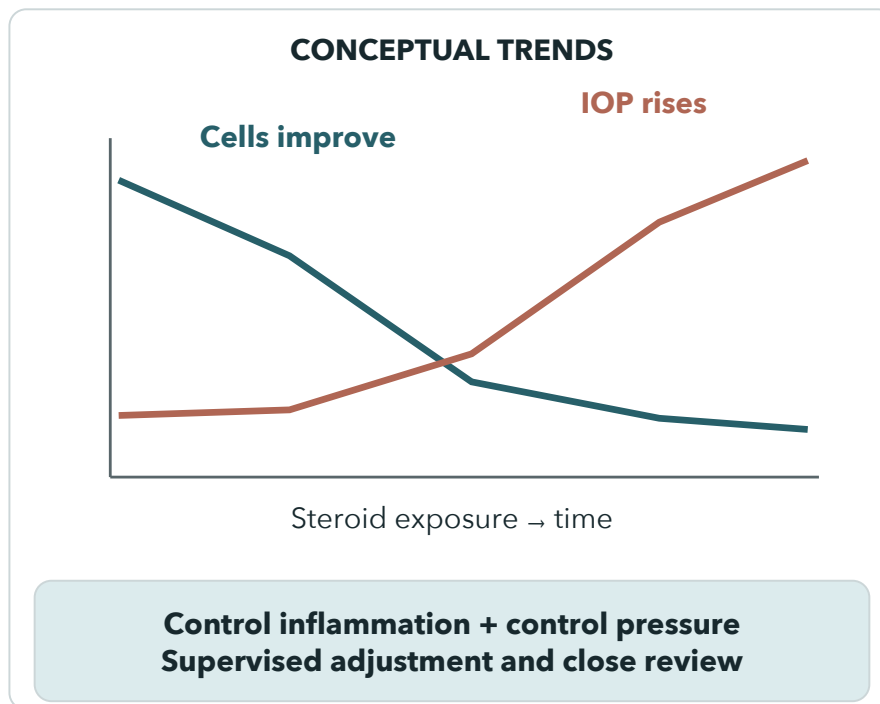


Inflammation with pressure rise or tissue loss

Protect the optic nerve while controlling inflammation; recognise destructive scleral disease.

Pressure and inflammation need separate checks

A | Two trends, two risks



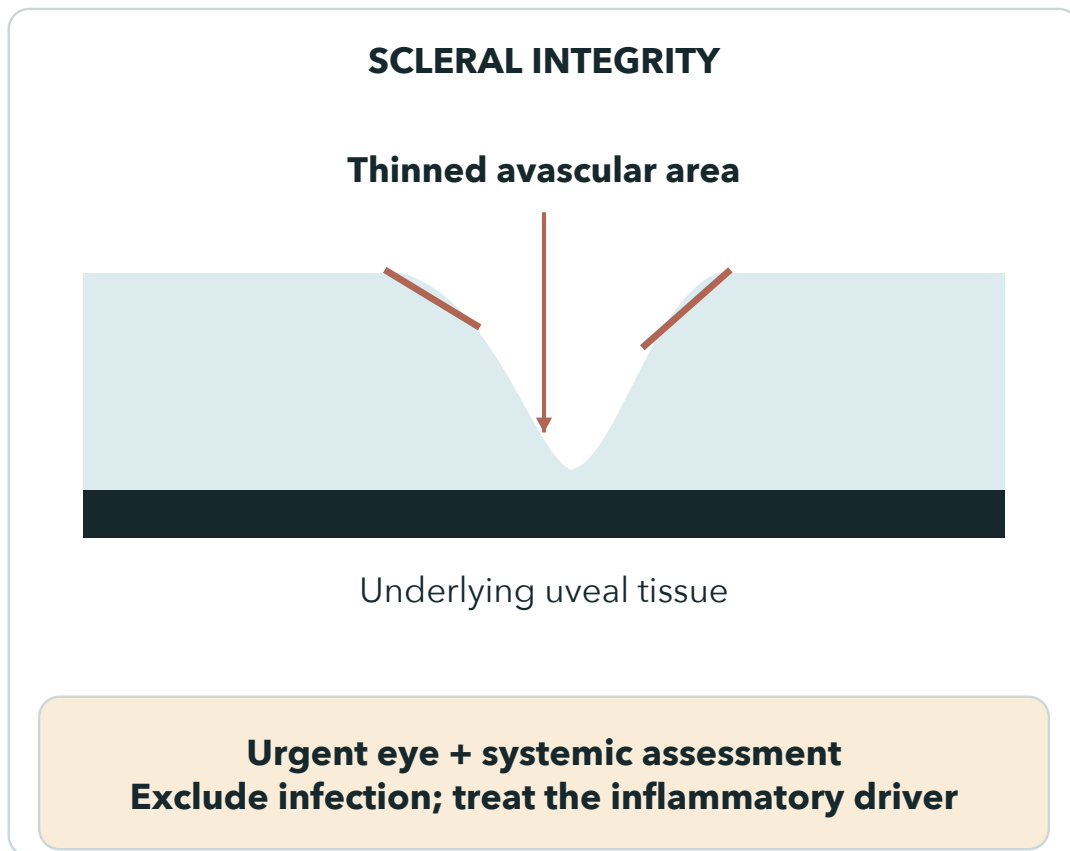
Original conceptual graph: chamber cells improve while pressure rises after steroid exposure. Values are illustrative, not treatment thresholds.

Reassess the mechanism

Possibility	What to review
Steroid response	Exposure, timing and pressure trend; modify treatment when safe.
Inflammatory outflow obstruction	Cells, trabeculitis and angle findings; inflammation still needs control.
Pupillary block	Synechiae, iris bombe and angle closure change the intervention.
Treatment plan	Supervised taper or safer regimen plus appropriate pressure control; monitor cells, IOP and nerve.

Thinning sclera signals a different level of risk

B | Necrosis threatens integrity



Original scleral section and escalation flow. The dark appearance through a thinned area reflects underlying tissue, not a superficial red patch.

Escalate systemic assessment

Necrotising scleritis can accompany serious rheumatoid or vasculitic disease even when joint symptoms are mild. Infection must be considered, especially with previous surgery, trauma or focal abscesses. Urgent ophthalmology and medical coordination guide systemic corticosteroid and immunomodulatory treatment for noninfectious disease. Topical drops or structural repair alone do not address the inflammatory driver. In uveitis, abrupt steroid cessation can provoke relapse; balance safe pressure lowering with sufficient inflammation control. If chamber cells improve while pressure rises after weeks of steroid, review the cause and adjust both treatments. Do not abruptly withdraw necessary anti-inflammatory therapy solely to improve the pressure reading. Conversely, do not accept prolonged high pressure simply because the redness has settled.